

Indy Inaugural '26 BRAINSTORM CANCER & COGNITION



Symposium Date: Saturday, September 5TH, 2026

*(SEPTEMBER IS **WORLD ALZHEIMER'S DISEASE MONTH**)*

Location: Sheraton Indianapolis Hotel at Keystone Crossing

8787 Keystone Crossing, Indianapolis, IN 46240

**THEME: PRESERVING COGNITION /S LIVING!
THINKING AS CLEAR AS A BELL**



DR. GATSON



DR. BROSCH

Living Oncology 501(c)(3)



BRAINSTORM

HONORING: KATHY LANE (left), RICHARD MANN (ctr), & JANE SCHNEIDER (right)



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Table of Contents

LIVING ONCOLOGY'S INDY INAUGURAL BRAINSTORM CANCER & COGNITION SYMPOSIUM – INDIANAPOLIS, IN 2026	F-COVER
HONORING THE LIVING OF KATHY LANE	2
HONORING THE MEMORY OF RICHARD MANN	2
HONORING THE LEGACY OF JANE SCHNEIDER	2
TABLE OF CONTENTS	3
ABOUT LIVING ONCOLOGY	4
BREAKFAST - LUNCH – DESSERT MENUS	5
MAIN SCIENTIFIC PROGRAM AGENDA	6
INNOVATOR SPEED ROUNDS (GET TO KNOW THE SCIENTISTS)	7
OPEN MIC FOR PATIENT 3-5 MIN TESTIMONIES FROM THEIR JOURNEY	7
CLOSING REMARKS & OPEN FOR LUNCH	7
PROVIDER NETWORKING EVENT	7
LIVING ONCOLOGY'S NEXT STEPS	8
FLOOR PLAN AND LAYOUT FOR INNOVATOR TABLES	9
SPONSOR ACKNOWLEDGEMENTS	10
DONOR ACKNOWLEDGEMENT & SPECIAL THANKS	11
GOLD SPONSORS – CARIS, NOVOCURE, IUHF, IUSM	11-15
KEY SPONSORS FOR LIVING ONCOLOGY AUTOMATION	16-17
PLATINUM SPONSOR – JAZZ PHARMACEUTICALS	18-19
DIAMOND SPONSOR – GT MEDICAL TECHNOLOGIES	20-21
DIAMOND SPONSOR – MENARINI SILICON BIOSYSTEMS	22-25
LIVING ONCOLO-TREE – DON'T FORGET TO LEAVE YOUR MARK	26
LIVING ONCOLOGY FLYER	27
DR. TOSHA'S UPCOMING BOOK SERIES	B-COVER
HEALTH CARE INSTITUTIONS INVOLVED IN THIS INAUGURAL SYMPOSIUM: BEACON HEALTH SYSTEM * COMMUNITY HEALTH NETWORK * FRANCISCAN HEALTH * GOODMAN CAMPBELL BRAIN & SPINE * INDY HEMATOLOGY EDUCATION * INDIANA UNIVERSITY HEALTH * PARKVIEW MEMORIAL HOSPITAL * UNION HEALTH	

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ABOUT LIVING ONCOLOGY



Dr. Gatson

I am **Dr. Na Tosha Gatson**, Founding President & CEO of Living Oncology, 501(c)(3) and a Neuro-Oncologist. **LO** is a charitable organization standing in the gaps that exist between the patient, the physician/provider, and the science. We serve as a neutral medium for the transfer of vital information that is critical to best patient outcomes. **We Believe That – LIVING is larger than life!**

This statement reflects our philosophy that quality of life and literacy empowerment are just as vital as medical treatment.

Oncology Knowledge: Collection of established facts about cancer treatment and patient outcomes. This can be applied or not applied.

Oncology Information: Timely updates that guide how cancer *knowledge* is applied with the goal to improve patient awareness.

Oncology Wisdom: The accumulation of knowledge and experience, balanced by the ability to access, interpret, and apply new information for best patient outcomes in cancer care.

Living Oncology's IMPACT:

To fuel the dispensation of information, with a goal to improve the cancer knowledgebase and grow the wisdom of the field. We are aiming for true health literacy, and not just patient education. **Health Education** teaches patients about their condition. **Health Literacy** provides the tools and resources to make informed cancer care decisions that lead to improved health outcomes!

#ChooseHealthLiteracy

Living Oncology's MISSION:

To improve health literacy by reconnecting patients and their caregivers to the scientific resources that allow for optimal **LIVING** through their cancer journey.

Living Oncology's VISION:

We envision a world where every person affected by cancer has access to comprehensive, understandable, and actionable information about their condition.



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Event Menu

Breakfast Menu (8am to 9am)

Assorted Bagels & Cream Cheese

Fresh Cut & Whole Fruit Display

Mango, Baby Papaya, Figs, Cantaloupe, Red and Black Grapes, Kiwi, Cantaloupe, Seedless Watermelon, Strawberries, Pineapple, Honeydew, Seasonal Berries & Honey Yogurt Sauce

Water & Iced Tea

VIVERE LUNCHEON – Monteris & Fragrances by Gé

Lunch Menu (Heavy HORS D'OEUVRES – 12:15pm to 2pm)

FOR THE VEGAN:

Caribbean Risotto Cakes

FOR THE VEGGIE:

Domestic and International Cheeses: *Maytag Bleu, Gorgonzola, Sharp Cheddar, Smoked Gouda, Boursin, Brie, Camembert, Pepper Jack, Havarti, Manchego and Fontina Cheeses, Garnished with Fruits and Berries, Crostini and French Baguette

Vegetable Spring Rolls

Mac & Cheese Bites

FOR THE OMMMMNIVORE:

Italian Antipasta: *Prosciutto, Soppressata, Genoa Salami, Capicola, Gourmet Olives, Roasted Artichoke Hearts, Peperoncini, House-Made Giardiniera, Fresh Mozzarella, and Provolone Served with Crusty Italian Bread and Flavored Italian Oils

Swedish Meatballs

Buffalo Chicken Wings

DESSERTS BY SIMPLY CAKES, "I'VE GOT WHAT IT CAKES": strawberry cupcakes, cheesecake shooters, red velvet blondies, fudge brownies, and kitchen sink blondies

Richard Mann. Jane Schneider. & Kathy Lane

BRAINSTORM CANCER IN '26

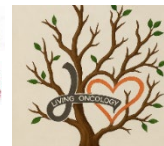
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CARE GIVING IS LIVING!

SCHEDULE: Sat, Sept 5th 2026

- 6:30 AM - 8 AM (1.5 hours)
INNOVATOR SET UP
Scientist/Innovator welcomed early to set up their booths. Scrolling slide show of patient photos.

- 8 AM – 9:00 AM (1 hour)
Open Registration, Visit the Innovators, & Enjoy Refreshments
Collect your name badge. Patients collect a symposium gift, Meet Innovators. **ADD YOUR THUMBPRINT TO THE LIVING ONCOLO-TREE**



- 9:00 AM – 9:20 AM (20 minutes)
WELCOME: 1st Annual Living Oncology - BrainStorm Cancer & Cognition IN '26
 - Living Oncology 501(c)(3) Impact: **LIVING POEM & PATIENT STORY – Leptomenigeal Carcinomatosis (LMC) Kathy Lane's Story**: Pivot for LIVING, Collaborative Medicine, the Value of Liquid Biopsy (**MenariniSearch/CellSearch**) – The Gift of More Time. (Menarini Silicone Biosystems)

- 9:20 AM – 10:20 AM (60 min)
Session 1: Brain Tumor Basics + LIVING Stories
Topics covered include 6-minutes each (Lecture + LIVING Story):
 - Dr. Jared Brosch** – "What is Cognition & Why and How Do We Preserve it?"
 - Dr. Pablo Bedano** – "What is the Value of Collaborative Care in Cancer Medicine?"
 - Dr. Girish Kunapareddy** – "Who serves on the Multidisciplinary Cancer Care Team?"
 - Dr. Josue Ordaz** – "What is the value of Neurosurgery & When NOT to Cut?"
 - Dr. Marwan Majeed** – "What is the Role for Pathology in Cancer Medicine?"
 - Dr. Sandeep Bhawe** – "Radiation Oncology Therapies Role & Potential Impact on Cognition"
 - Dr. Adam Comer** – "Importance of Communication: Dealing with a Terminal Illness"
 - Chaplains Vern Farnum & Missi Koenig** – "Importance of Spiritual Wellness in Patient Care"
 - Lauren Perry-Moore** – "Why is Research Important to My Care"
 - AH's Story** – "Deciding Care WITH your Doctor"

- 10:20 AM - 10:45 AM (20 min) – FIVE 5-MIN JOURNEYS

SPONSORED SPEAK EASY SESSION

THE JOURNEY IS NOT ONE – IT IS WON!

- | | |
|-------|---|
| 10:20 | <ul style="list-style-type: none">Jane Schneider's Story (Choosing LIVING, Driven by Family, Standing on Faith)<ul style="list-style-type: none">Value of Tissue Pathology (GT Med Tech; Caris Life Sciences; Novocure) |
| 10:25 | <ul style="list-style-type: none">Richard Mann's Story (He LIVES in our Hearts)<ul style="list-style-type: none">Value of Cognitive Care - Dr. Jared Brosch to tell the story of a sister's love. |
| 10:30 | <ul style="list-style-type: none">Joey's Story (How Do you Defy the Stats? LIVING! Something's Gotta Give!)<ul style="list-style-type: none">Value of Communicating and being HEARD (Novocure – Optune Gio) |
| 10:35 | <ul style="list-style-type: none">Danny Efron's Story (Prioritizing Living & Teaching Us How to Tell Our Own Story)<ul style="list-style-type: none">Value of Connecting (GT Medical Technologies – GammaTile) |

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- 10:40 AM – 10:55 AM (15 minutes)

Biological Break and Visit Scientists

ADD YOUR THUMBPRINT TO THE LIVING ONCOLO-TREE



LEAVE YOUR MARK



10:55p – 11:05p (INNOVATOR SPEED ROUNDS)

MEET THE 14 INNOVATORS (45 seconds SPEED ROUNDS) Pass the Mic

- 11:05 AM – 12:05 PM **(Patient stories – 3-5 Minute Mic Drop)**

(A Testimony from your Journey – Open to Patients/Caregivers)

EACH PRESENTER WILL BE HONORED WITH A SMALL TOKEN

- Story One
- Story Two
- Story Three
- Story Four
- Story Five
- Story Six
- Story Seven

12:05 – 12:10PM (5 minutes) **CLOSING REMARKS – DISMISS THE PATIENTS & FAMILIES FOR LUNCH**

- 12:15 – 2 PM**

Monteris Medical & Fragrances by Gé Sponsored luncheon

(See lunch menu (page 5))

ANTHONY STARACE VIVERE LUNCHEON

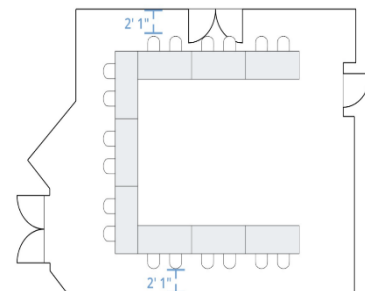
Socializing, Networking, Visit the Scientists, & LUNCH

**PROVIDER NETWORKING LUNCHEON
& HONORARIUM IN CROSSPOINTE BOARDROOM**

PROVIDER & SCIENTIST NETWORKING (12:15 – 1:30p)

BOARD ROOM SCIENTIFIC PRESENTERS:

- Menarini Silicon Biosystems (20 min) 2 reps
- GT Medical Technologies (20 min) 2 reps
- Jazz Pharmaceuticals (15 min) 1 rep
- Novocure (7 min) 1 rep
- Caris Life Sciences (7 min) 1 rep



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Next Steps For LIVING ONCOLOGY. Where Do We Go From Here? Patient-facing science is a patient's right. We must always allow for ample opportunities to connect with the science. Making informed decisions about their care depends on it! Reconnecting might also guide and improve patient clinical trial participation, rebuild trust in medicine, and stimulate better collaboration amongst providers.



Check out www.NeurologyLive.com:



Special Episode: Na Tosha, Gatson, MD, PhD, on Newly Launched Neuro-Oncology Column

July 30th 2026

Marco Meglio, Na Tosha Gatson, MD, PhD, FAAN

Mind Moments®, a podcast from NeurologyLive®, brings you an exclusive interview with neuro-oncologist Na Tosha Gatson, MD, PhD, FAAN. [LISTEN TIME: 20 minutes]

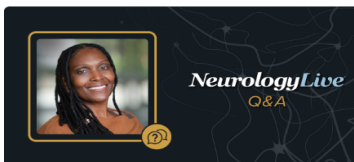


Introducing The G.L.O.B: Dr. Gatson's Living Oncology Brainstorm Column

July 8th 2026

Na Tosha Gatson, MD, PhD, FAAN

Led by Na Tosha Gatson, MD, PhD, FAAN, director of the Center for Neuro-Oncology at the Indiana University Health Neurosciences and Cancer Institutes, this new column translates complex neuro-oncology topics into accessible, evidence-based insights.



From Trial Design to Institutional Policy: Advancing Equity in Brain Tumor Care

February 27th 2026

Marco Meglio

In honor of Black History Month, neurologist Na Tosha Gatson, MD, PhD, reflects on structural inequities in neuro-oncology and advancing equity through mentorship, trial design, and institutional policy.

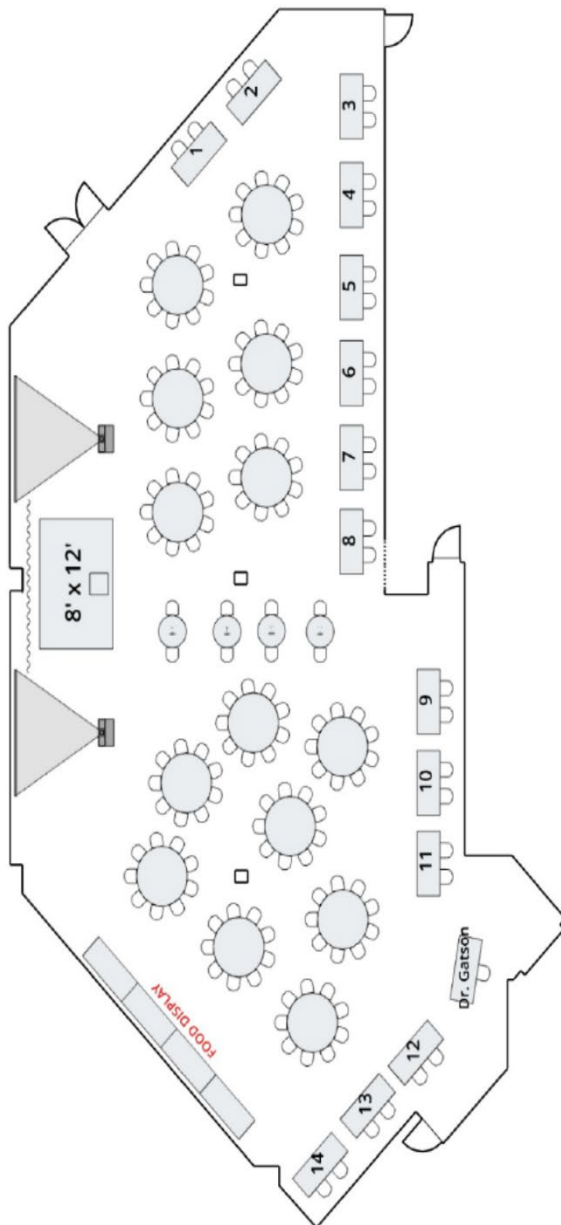
NEXT UP: BrainStorm Cancer Symposium - May 8, 2027 - AZ

Ultimately, I hope it BRAINSTORMS in every state!

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Floor Plan (Clearwater Ballroom: Vendor Tables size 6' x 30" w/2 chairs)

LIST OF SCIENTIFIC CONTRIBUTORS & INNOVATORS (TABLE #)



RECTANGULAR TABLES:

1. **Menarini Silicon Biosystems** (Liquid Biopsy)
(Focused on non-invasive cell-based diagnostics, single-cell precision, and advancing personalized medicine)
2. **GT Medical Technologies (GammaTile)**
“Dedicated to improving the lives of patients living with operable brain tumors”
3. **Jazz Pharmaceuticals** (Modeyso - midline glioma)
(Our purpose is to innovate to transform the lives of patients and their families)
4. **Caris Life Sciences**
“Fulfilling the Promise of Precision Medicine”
5. **Novocure** (Optune Gio, TTFields)
“Together with our patients, we strive to extend survival in some of the most aggressive forms of cancer”
6. **(OPEN) Monteris Medical** (LITT Surgery)
“We Deliver Hope and Healing”
7. **Community Health Network MDACC**
“Exceptional Care. Simply Delivered”
8. **RESERVED FOR CAMERA**
9. **IU Health Foundation – Master Clinician**
“Recruitment of Visionaries” Pharmacy Partner
10. **Indiana University School of Medicine**
“Department of Gift Development w/Sam Kinder
11. **Shar’s Wigs (Shar Brown)**
“The Gift of Preserving Self & Identity Wellness”
12. **American Brain Tumor Association**
(Advance the understanding & treatment of brain tumors)
13. **Fragrances by Gé (Glenn Caruthers)**
“Smell Good. Feel Seen.”
14. **Union Health**
“Healthier, together.”

LIVING ONCOLOGY TABLE – DR. GATSON

“LIVING is larger than Life.”

Sponsor Acknowledgements

DIAMOND Sponsors:

Menarini Silicon Biosystems (4 Events)

GT Medical Technologies (4 Events)

PLATINUM Sponsor:

Jazz Pharmaceuticals (4 Events)

Networking Event Sponsors:

Menarini Silicone Biosystems, GT Medical Technologies, Jazz Pharmaceuticals, Novocure, & Caris Life Sciences

GOLD Sponsors:

Caris Life Sciences (4 Events)

Novocure (4 Events)

Indiana University Health Foundation (2 Events)

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VIVERE PATIENT LUNCHEON:

Monteris (2 Events)

Fragrances by Gé (1st Event)

DONORS OTHER Support:

Nefer Kali Healing (4 Events)

Cost Avoidance Automation Kingz Enterprises (4 Events)

American Brain Tumor Association (3 Events)

Shar's Wigs (2 Events)

IU School of Medicine Department of Gift Development

- Sam Kinder (1st Event)



Special Thanks to Jenna Keisler of **SHERATON INDIANAPOLIS KEYSTONE CROSSING** and Joshua Flint of **ENCORE GLOBAL** for helping to make this event possible and for their willingness to meet LO at the finish-line! We are so excited to have worked with a team that cares for people TOO.



Special Thanks to all of the patients, caregivers, families, physicians, providers, learners, and scientists/innovators who aim to end cancer and work hard daily to **PRESERVE COGNITION**.



Special Thanks to **Dr. Jared Brosch** for his willingness to be the Inaugural presenter for BrainStorm **Cognition** Awareness.



Special Thanks to Ucheoma Eze & the **LO Team & CAAKE** for making **AUTOMATION** Simple (as easy as CAAKE)!





SCHOOL OF MEDICINE
DEPARTMENT OF NEUROLOGY



Foundation

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patientforward

Together with our patients, we strive to extend survival in some of the most aggressive forms of cancer by developing and commercializing our innovative therapy.

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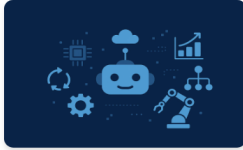
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The HEALTHCARE AUTOMATION Study

AI-Powered Solutions for Healthcare Administrative Efficiency

CAAKE

Cost Avoidance Automation Kingz Enterprise



- Healthcare administrative tasks consume over 25% of total healthcare spending in the United States
- CAAKE's AI automation solutions can reduce administrative workload by up to 37% for healthcare providers
- Our intelligent systems integrate with existing EMR/EHR systems, reducing training time and improving adoption rates

The HEALTHCARE AUTOMATION study is a comprehensive analysis of AI implementation in healthcare administrative processes, demonstrating how CAAKE's solutions increased patient satisfaction by 29% and saved over 2,000 staff hours monthly for Meridian Health Group.

Ideal Implementation Scenarios

- Healthcare providers with 10+ administrative staff
- Practices experiencing patient scheduling bottlenecks
- Organizations seeking to optimize insurance verification processes
- Medical groups wanting to reduce documentation time for clinicians
- Healthcare systems looking to implement patient engagement solutions
- Not sure? Contact us for a free assessment of your automation potential

Implementation Options (3-6 months typical timeline)

AI Assistants

Virtual staff augmentation

Workflow Automation

Process optimization

Custom Integration

Comprehensive solution

Implementation strategy will be customized based on your specific needs and existing technology infrastructure.
Organizations of all sizes can benefit from our scalable solutions.

Primary Outcomes:

- Efficiency: Reduced administrative workload and improved staff productivity
- Patient Experience: Improved satisfaction scores and engagement
- ROI: Measurable cost savings and resource optimization

Implementation Status:

- CAAKE automation solutions are currently deployed in over 120 healthcare facilities globally
- Case studies and testimonials from satisfied clients are available at caake.org/case-studies
- Funding and financing options are available for eligible healthcare organizations; contact sales@caake.org for more information

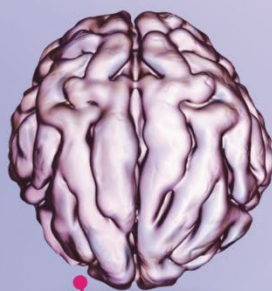
CAAKE's AI automation solutions are customizable and can be integrated with most major healthcare systems and software. Implementation timelines and results may vary based on specific organizational requirements and existing infrastructure.

References

1. Meridian Health Group Case Study: 37% reduction in administrative tasks, 29% improvement in patient satisfaction
2. JAMA Network: "Administrative costs account for 34.2% of healthcare expenditures in the United States"



GLIOMA



TEST ALL. TEST EARLY.

TEST FOR H3 K27M

H3 K27M–mutant diffuse gliomas can occur across the brain, in both midline and non-midline structures, and are characterized by the global loss of histone H3 K27 trimethylation.^{1,2}

Prognosis is poor for patients with H3 K27M–mutant diffuse glioma, with a **median overall survival of approximately 1 year from diagnosis.**³⁻⁵

EARLY MOLECULAR TESTING IS ESSENTIAL FOR PROPER DIAGNOSIS⁶⁻⁸

A biopsy with subsequent immunohistochemistry (IHC) or next-generation sequencing (NGS) reveals specific mutations, such as H3 K27M, and can provide additional data to inform disease management.⁶⁻⁸

TEST ALL. TEST EARLY. TEST FOR H3 K27M

Scan the QR code or visit **TestForH3K27M.com** to learn more



References: 1. Saratsis AM, Knowles T, Petrovic A, Nazarian J. H3K27M mutant glioma: disease definition and biological underpinnings. *Neuro Oncol.* 2024;26(suppl 2):S92-S100. doi:10.1093/neuonc/noad164 2. Nakatogawa H, Fukai J, Kawaji H, et al. Characteristics and outcomes of diffuse non-midline gliomas with H3F3A gene mutation in the Kansai Molecular Diagnosis Network for CNS tumors (Kansai Network): multicenter retrospective cohort study. *Acta Neuropathol Commun.* 2025;13(1):77. doi:10.1186/s40478-025-01989-y 3. Vuong HG, Ngo TNM, Le HT, et al. Prognostic implication of patient age in H3K27M-mutant midline gliomas. *Front Oncol.* 2022;12:858148. doi:10.3389/fonc.2022.858148 4. Ostrom QT, Shoaf ML, Cioffi G, et al. National-level overall survival patterns for molecularly-defined diffuse glioma types in the United States. *Neuro Oncol.* 2023;25(4):799-807. doi:10.1093/neuonc/noac198 5. Zheng L, Gong J, Yu T, et al. Diffuse midline gliomas with histone H3 K27M mutation in adults and children: a retrospective series of 164 cases. *Am J Surg Pathol.* 2022;46(6):863-871. doi:10.1097/PAS.0000000000001897 6. Horbinski C, Solomon DA, Lukas RV, et al. Molecular testing for the World Health Organization classification of central nervous system tumors: a review. *JAMA Oncol.* 2025;11(3):317-328. doi:10.1001/jamaoncol.2024.5506 7. Horbinski C, Nabors LB, Portnow J, et al. Central nervous system cancers, version 2.2022: featured updates to the NCCN Guidelines. *J Natl Compr Canc Netw.* 2023;21(1):12-20. doi:10.6004/jnccn.2023.0002 8. Gajjar A, Mahajan A, Bale T, et al. Pediatric central nervous system cancers, version 2.2025. *J Natl Compr Canc Netw.* 2025;23(3):113-130. doi:10.6004/jnccn.2025.0012



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Discover **immediate radiation treatment** with GammaTile®

Designed for operable brain tumors*, GammaTile is a form of radiation therapy placed at the time of tumor removal surgery. **It starts working right away – and continues working while you heal.¹**

- **Immediate:** No radiation treatment delay¹
- **Targeted:** Designed to protect healthy brain tissue¹
- **Controlled:** Uniform radiation delivered directly to the tumor site¹

*GammaTile is indicated as a treatment for patients with newly diagnosed malignant (cancerous) brain tumors and patients with recurrent brain tumors.

Is GammaTile right for you?

☒ I have a brain tumor that can be removed with surgery.

Note: GammaTile is designed for patients with operable brain tumors.* If this applies to you, continue below:

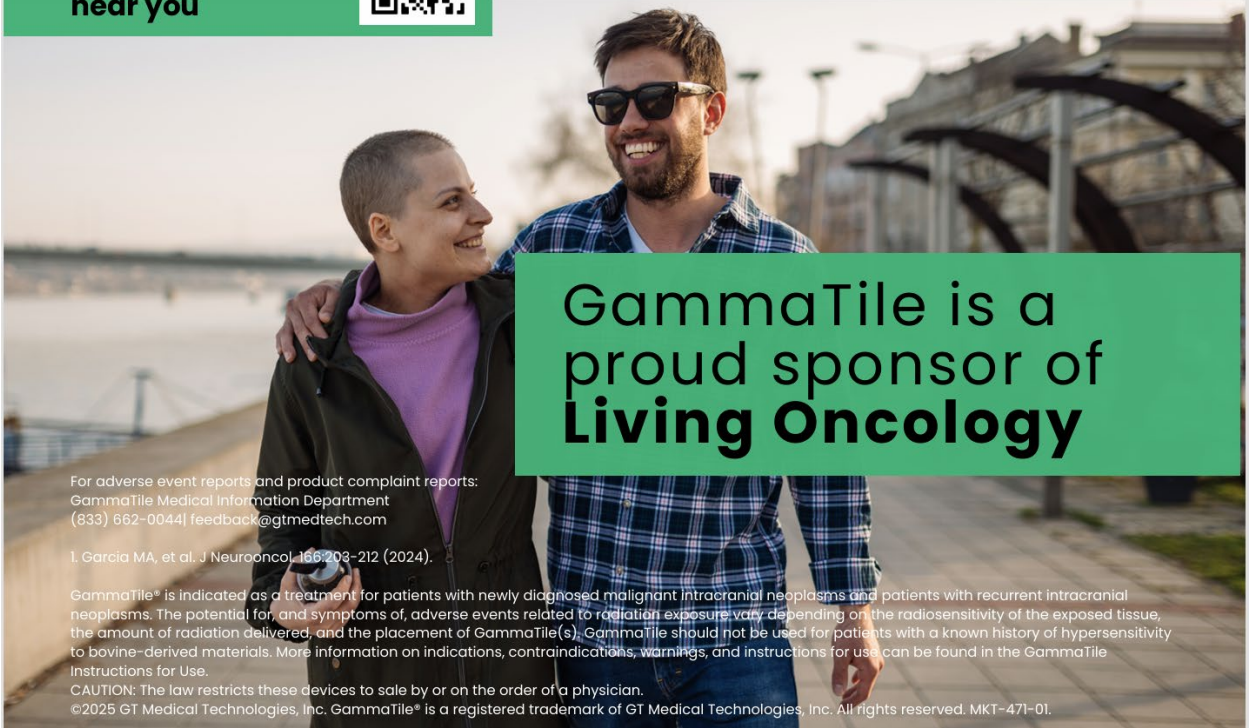
Check any statements that apply to you.

- ☐ I want radiation treatment that begins right at the time of surgery, with no delay.
- ☐ It would be difficult for me to return to the hospital for follow-up radiation.
- ☐ I live far away or don't have easy access to a radiation center.
- ☐ I am concerned about hair loss from radiation treatment.
- ☐ My tumor has come back or spread to a different part of my brain after previous treatment.

Next Steps:

If you selected **one or more** of the options above, you may benefit from GammaTile and we encourage you to talk with your doctor about whether GammaTile may be right for you.

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1. Garcia MA, et al. J Neurooncol. 166:203-212 (2024).

GammaTile® is indicated as a treatment for patients with newly diagnosed malignant intracranial neoplasms and patients with recurrent intracranial neoplasms. The potential for, and symptoms of, adverse events related to radiation exposure vary depending on the radiosensitivity of the exposed tissue, the amount of radiation delivered, and the placement of GammaTile(s). GammaTile should not be used for patients with a known history of hypersensitivity to bovine-derived materials. More information on indications, contraindications, warnings, and instructions for use can be found in the GammaTile Instructions for Use.

CAUTION: The law restricts these devices to sale by or on the order of a physician.

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environment



Gene
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RNA
Mutations



Virus



Immune
Response



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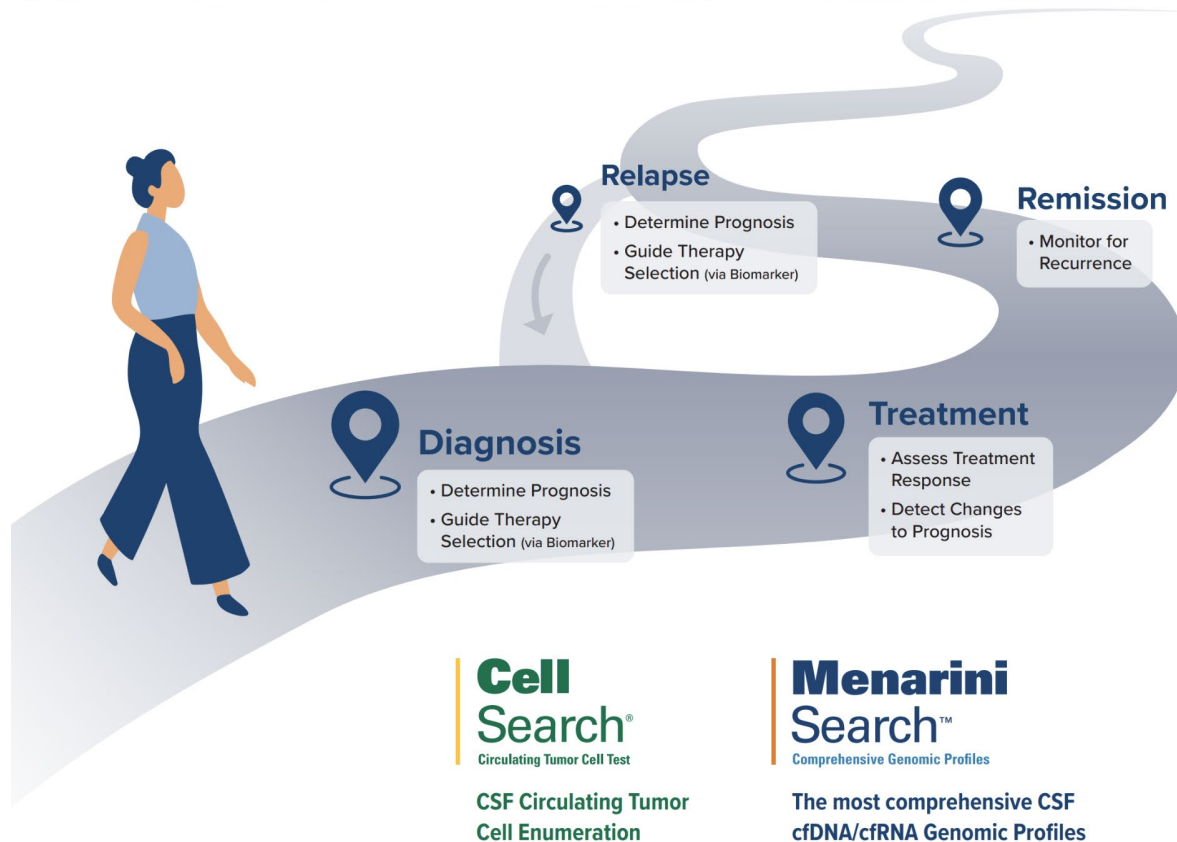
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Remarkable progress has been achieved in various aspects of modern medicine, but early diagnosis and effective treatment of brain tumors are still challenging.

Conventional diagnostic methods like neuroimaging are expensive, complex, and can only provide limited information for therapeutic benefits. Comprehensive Genomic Profiling (CGP), particularly liquid biopsy, is increasingly being adopted across most cancer types. Constant monitoring of key biomarkers can be crucial in managing brain tumors, guiding therapy, and improving outcomes by allowing for timely and targeted interventions.

What is Comprehensive Genomic Profiling?

Comprehensive Genomic Profiling (CGP) is a cutting-edge approach in cancer treatment that analyzes the DNA and RNA from your cancer cells. This testing helps identify unique markers and mutations in cancer cells, allowing for highly personalized treatment strategies.

What are cfDNA and cfRNA?

Cell-free DNA (cfDNA): These are tiny fragments of DNA shed from cancer cells into your bloodstream. Analyzing these fragments helps identify the genetic mutations your cancer cells carry. Cell-free RNA (cfRNA): Similar to cfDNA, cfRNA consists of RNA fragments that provide crucial information about which genes are active and how they are controlled in your cancer cells.

What is a Liquid Biopsy?

A liquid biopsy is a simple and minimally invasive test that requires only a sample of your blood or cerebrospinal fluid (CSF). Lumbar puncture is the routine extraction approach for CSF-based tests. This sample is used to extract cfDNA and cfRNA, which are then analyzed to understand the behavior of your cancer.

With the recent advances in liquid biopsies, patients benefit from more genetic information, which can help them with targeted or personalized treatment options. Liquid biopsy can be performed regularly to monitor the disease process, treatment effect, and prediction of relapse.

<https://www.frontiersin.org/journals/genetics/articles/10.3389/fgene.2021.720270/full>

What is Cerebrospinal Fluid?

Cerebrospinal fluid (CSF) surrounds and cushions the brain and spinal cord. It performs vital functions, including providing nourishment, waste removal, and protection to the brain.

<https://www.ncbi.nlm.nih.gov/books/NBK519007/>

Why CSF Testing?

Testing the CSF helps diagnose various conditions, including cancers that affect the central nervous system or metastasize to these regions. Certain tests can detect cancer cells, DNA, or other cancer markers that may have spread to the brain or spinal column. This is crucial for accurate staging and treatment planning.

1. Proximity to the Disease Site: CSF is physically closer to brain tissues and may reflect tumor burden more directly. In brain tumors, CSF proves to be a better source of tumor-derived cfDNA than plasma since the concentration of circulating tumor DNA is higher <https://www.frontiersin.org/journals/genetics/articles/10.3389/fgene.2021.720270/full>
2. Biomarker Preservation: CSF may better preserve certain biomarkers that degrade more rapidly or are diluted in the bloodstream. This is particularly relevant for sensitive molecular analyses, like detecting specific mutations or changes in cfDNA or cfRNA that can indicate the presence or progression of a tumor.
3. Disease Progression and Treatment Response: Changes in the levels or composition of cfDNA and cfRNA assessed from the CSF sample can indicate how effectively a tumor responds to treatment, allowing adjustments to be made more swiftly than waiting for physical symptoms or imaging studies.



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How Liquid Biopsies Work?

A liquid biopsy is a test done on a sample of peripheral blood or CSF to look for cancer cells or pieces of DNA and RNA from tumor cells.

What to Expect After the CSF Collection

CSF collection is an outpatient procedure. Patients might need to stay in the hospital for a few hours for monitoring. Results are typically available within a few weeks, depending on the specific tests performed.

Empowering Your Journey: The Patient's Role in CSF Testing

Questions to ask your doctor:

- 1 What specific tests will be done on my CSF sample?
- 2 How will the cfDNA and cfRNA results influence my treatment plan?
- 3 What follow-up or additional tests might be necessary?

What Patients Should Know About CSF Collection Procedures

Procedure Overview:

The procedure to collect CSF is called a lumbar puncture or spinal tap, where a needle is used to collect fluid from the spinal column. It is generally safe, with minimal risks such as headache, infection, or bleeding. It is less invasive than traditional tissue biopsies and can provide much information about the cancer.

Advocating for Comprehensive Testing

Consider discussing with your oncologist the possibility of incorporating comprehensive genomic profiling into your treatment plan. Understanding your cancer's unique genetic makeup can open the door to optimized treatment strategies and improved outcomes.

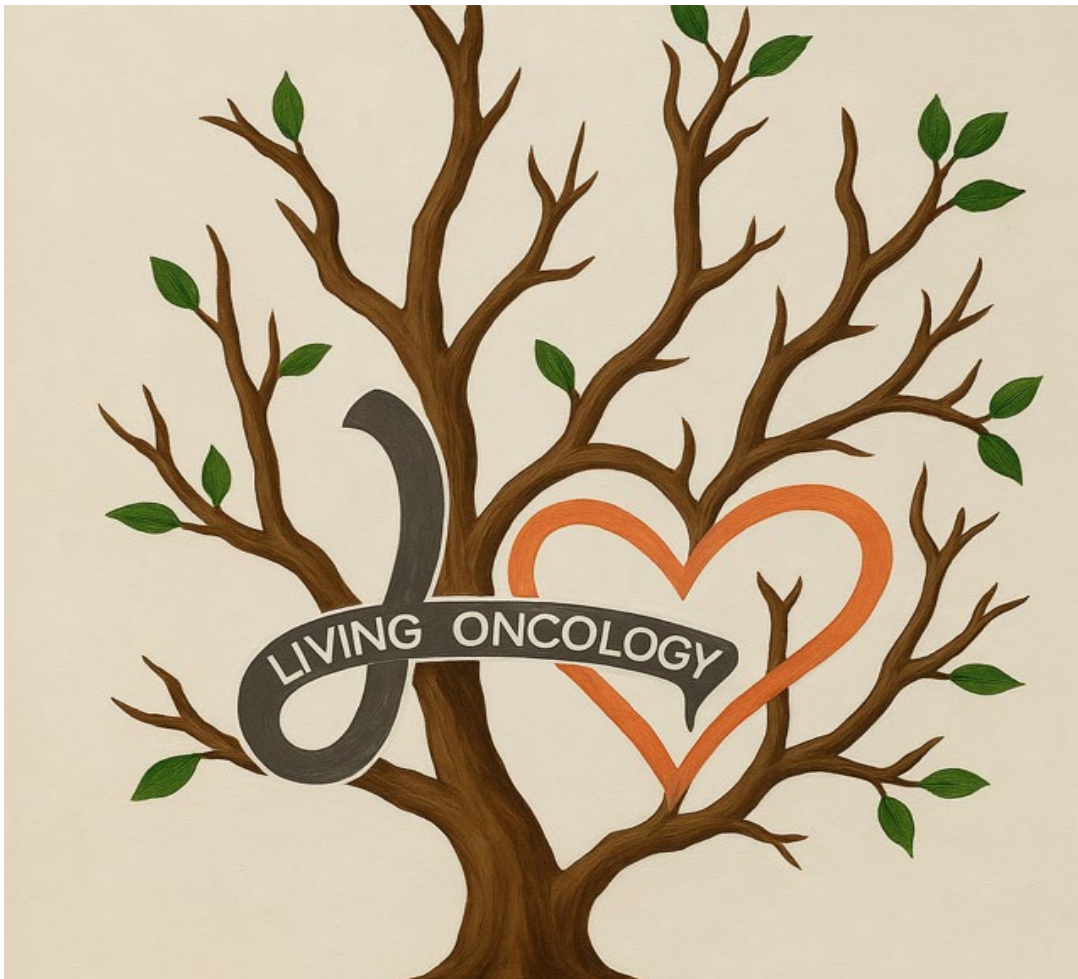
Also consider discussing the potential benefits of CSF testing with your healthcare team, especially if you have a cancer that could affect the brain or spinal cord.

Always consult with a healthcare professional to ensure the information is tailored to your specific condition and needs.



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LIVING is larger than Life.

Meet Our Founder



Dr. Na Tosha Gatson, MD, PhD, FAAN

is a neuro-oncology specialist and founder of Living Oncology. She is committed to bridging the gap between complex cancer science and patient understanding, making advanced knowledge more accessible to all.



Our Mission

To improve health literacy by reconnecting patients and their caregivers to the scientific resources that allow for optimal LIVING through their cancer journey.



Our Vision

We envision a world where every person affected by cancer has access to comprehensive, understandable, and actionable information about their condition.

“LIVING is larger than Life” — reflecting our philosophy that quality of life and empowerment are as vital as medical treatment.



**Scan to learn more about
Living Oncology and our mission**



(602)292-5227



ThePresident@livingoncology.org

Dr. Tosha Book Series – The Glowing Brain: *I AM ON PURPOSE*



AGES 3-6

A Child: There are no mistakes



AGES 7-12

A Tween: A path has been laid before you.



AGES 13-17

A Teen: You are ON your designated path.



AGES 18+

A Vessel... You Are, With **Intentionality!**